



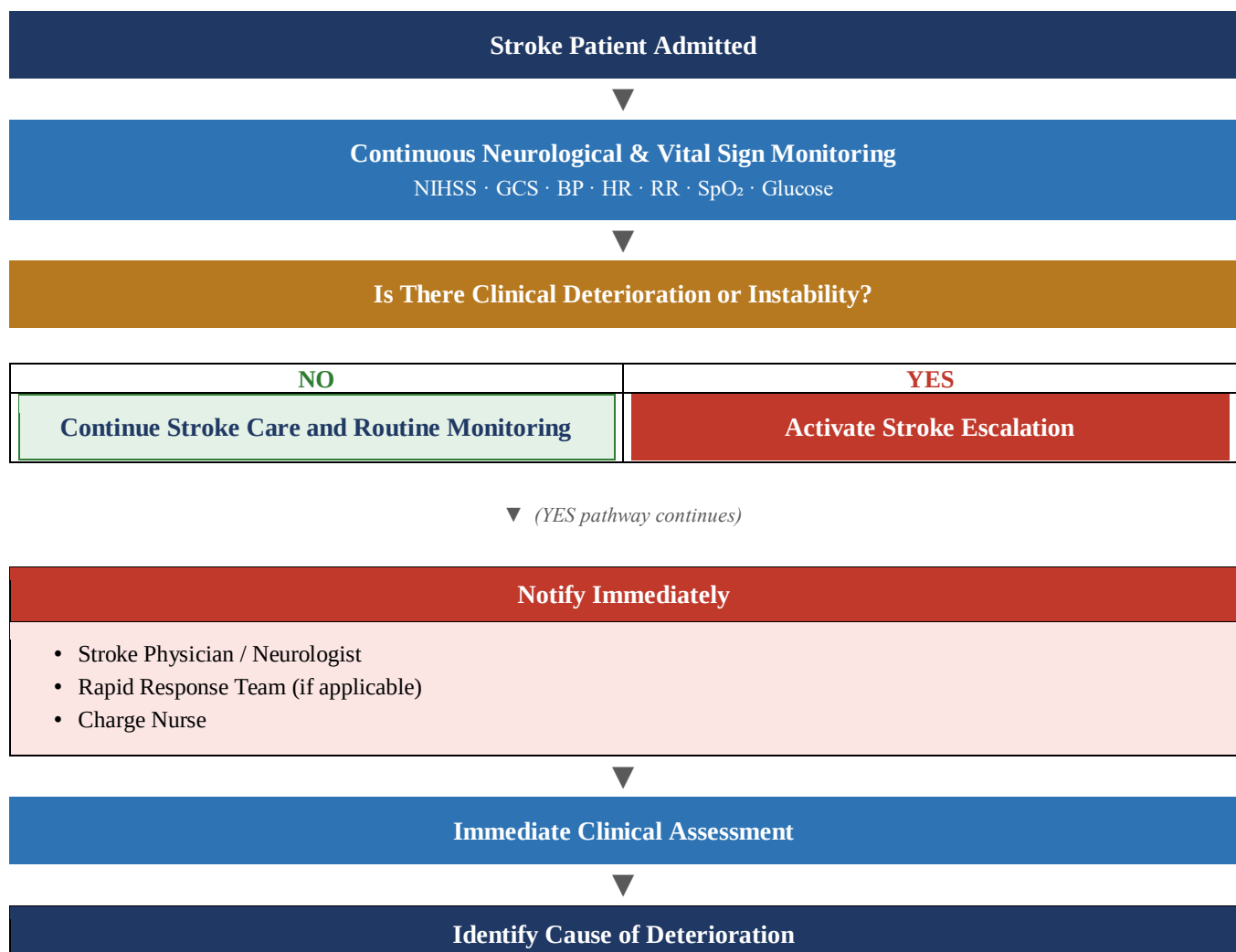
RAPID RESPONSE AND ICU TRANSFER FLOWCHART FOR STROKE PATIENTS

Stroke Unit — Clinical Escalation Pathway

Purpose

- To provide a standardized process for the rapid recognition, escalation, and transfer of stroke patients who develop neurological or medical deterioration requiring higher-level care.

Escalation Pathway



- Decline in GCS
- NIHSS increase ≥ 4
- New focal neurological deficit
- Airway compromise
- Respiratory failure
- Seizure
- Hemodynamic instability
- Hemorrhagic transformation
- Cerebral edema



Immediate Diagnostic Evaluation

- STAT CT Brain $\pm$ CTA
- Laboratory investigations
- ECG
- Blood glucose



Does the Patient Require Critical Care?

NO	YES
Continue Stroke Unit Treatment	Notify ICU Team · Prepare ICU Bed · Arrange Safe Transfer

▼ (YES pathway continues)

ICU Admission and Handover



Initiate ICU Stroke Management

- Airway protection
- Mechanical ventilation (if needed)
- ICP management
- Hemodynamic monitoring
- Hyperosmolar therapy
- Continuous neurological monitoring
- Neurosurgical consultation (if indicated)



Ongoing Reassessment

ICU Transfer Criteria

Transfer to the ICU should be considered for patients with one or more of the following:

- | |
|--|
| • Decreased level of consciousness (GCS $\leq$ 12 or rapidly declining) |
| • NIHSS worsening (e.g., increase $\geq$ 4 points) |
| • Airway compromise or need for intubation |
| • Respiratory failure |
| • Hemodynamic instability requiring vasoactive support |
| • Large hemispheric infarction with cerebral edema |
| • Intracerebral hemorrhage requiring intensive monitoring |
| • Hemorrhagic transformation after thrombolysis or thrombectomy |
| • Status epilepticus or recurrent seizures |
| • Need for invasive neurological or cardiovascular monitoring |
| • Requirement for hyperosmolar therapy or intracranial pressure management |
| • Immediate neurosurgical intervention |

Clinical Note

- This flowchart is intended to support, not replace, clinical judgment. All escalation decisions should be documented in the patient record, including time of notification and response.